

A patient presents on the day of surgery with a blood pressure of 180/110mmHg. Justify your decision to proceed with or postpone surgery for this patient.

Sample Answer

The stem provides insufficient information about the patient and nature of surgery. In such a situation, my decision would be to **postpone surgery** based on the following assumptions:

- (1) This is an elective surgery.
- (2) Blood pressure readings were repeated and verified to be non-spurious.

Justifications

My decision to postpone surgery is supported by the 2016 joint guidelines by AABGI and British Hypertension Society, which recommends a threshold of BP <180/110 mmHg before proceeding with elective surgery.

In addition, the elevated blood pressure of 180/110 mmHg in this patient could represent several scenarios where it would not be medically appropriate to proceed:

1. Hypertensive Urgency/ Emergency

- Priority would be to rule out and treat potential end-organ injuries such as stroke and AMI, which would increase peri-op mortality and morbidity if left untreated.

2. Poorly controlled Hypertension

- More labile haemodynamic profile, leading to greater fluctuations in BP and HR and increased risk of intra-op hypertension.
- Increased sympathetic response to intra-op stimulus, which can lead to high BP and increased bleeding.
- Associated with increased risk of atherosclerosis in major end organs – increasing risk of stroke, CAD, CKD, PVD. These co-morbidities should be worked up and optimised prior to surgery.

3. Newly diagnosed Hypertension

- These patients should start treatment and attain satisfactory BP control prior to surgery to reduce their risk of MACE events.
- Need to exclude secondary causes for hypertension if patient is young.

Given that surgery postponement comes with financial/ social/ psychological costs to hospital and patient, I will discuss with the surgeon to get his input. I will also explain and counsel my patient so that he may understand the rationale for surgery postponement .

Caveats

However, there are specific situations where benefits outweigh risks, and I may **proceed with surgery** after counselling for elevated MACE risks based on patient/ surgical risk profiles. These include:

1. Emergency (life/limb/eye-threatening conditions) or Time-sensitive (malignancy/ severe infection requiring urgent source control) surgeries
2. Surgeries for BP management (e.g. adrenalectomy for pheochromocytoma, caesarean section for parturient with pre-eclampsia)
3. Persistently high blood pressure despite compliance to medical treatment and optimization by medical team
4. White coat hypertension (normal home BP monitoring, with secondary causes of hypertension ruled out)